

Please read this leaflet carefully before you start using your **Budenase AQ**

6.0 ml

Budesonide Aqueous Nasal Spray BP 64 mcg



Budenase AQ

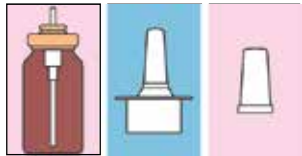
64 mcg /dose

120

METERED DOSES

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Patient Information
Parts of Budenase AQ



Bottle with Metered-Dose Atomising Pump Nozzle Dust Cap

Important

1. Use your Budenase AQ as directed by your physician. Do not exceed the recommended dose.
2. It takes a few days for this medicine to work hence it is very important that you use it regularly. Do not stop treatment even if you feel better unless told to do so by your doctor.
3. This medicine has been specially recommended by your doctor for you. Do not allow any other person to use it.
4. Keep your **Budenase AQ** out of the reach of children.
5. Protect your **Budenase AQ** from light.

How to use your Budenase AQ Before using

1. Shake the bottle gently then remove the dust cap.
2. Hold the bottle as shown with your forefinger and middle finger on either side of the nozzle and your thumb underneath the bottle.



3. Before using your Budenase AQ for the first time, or if you have not used it for a longer period, follow the instructions given below.

With the nozzle pointing away from you, press several times as shown until a fine uniform mist comes out of the nozzle.



4. Blow your nose gently.



5. Close one nostril and put the nozzle in the other nostril as shown. Tilt your head slightly forward keeping the nozzle in the upright position.

6. Start to breathe in through your nose and while breathing in press down with your fingers once to release a spray.



7. Breathe out through your mouth. Repeat steps 6 and 7 to inhale a second spray.



8. Repeat steps 5,6 and 7 for the other nostril



After use

9. Wipe the nozzle with a clean cloth and replace the dust cap.



Budenase AQ

Note

The first time you use you may sneeze a few times. This is because the lining of your nose is very sensitive. This is not likely to happen after the first few doses. This medication can, at times cause a few side-effects in certain hypersensitive individuals. If you notice any bleeding from your nose or experience any other discomfort, please inform your doctor.

Cleansing

Your Budenase AQ should be cleaned atleast once a week. To do this :



1. Push the nozzle upwards to detach from the bottle.
2. Wash the nozzle and dust cap in warm water. Shake off excess water and leave to dry in a warm place. Avoid excessive heat.
3. Replace the nozzle and dust cap.
4. If the nozzle becomes blocked remove as above and soak in warm water. Rinse under a cold tap. Allow to dry and refit. Do not use a pin or other sharp objects to unblock the nozzle.

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For the use only of a Registered Medical Practitioner or a Hospital or a Laboratory

Budenase AQ Nasal Spray
64 mcg/dose

(Budesonide Aqueous Nasal Spray BP 64 mcg 120 Metered sprays)

Each spray delivers
Budesonide BP / Ph. Eur 64 mcg
Composition :
Budesonide BP / Ph. Eur 0.128 % w/w
Potassium Sorbate BP / Ph. Eur 0.12% w/w
(Preservative)

Description

A white, off-white homogeneous, redispersible suspension filled in 10 ml amber glass vial with crimped nasal spray assembly

Pharmacology

Pharmacodynamics

Budesonide is a non-halogenated glucocorticosteroid with a high local anti-inflammatory action within the respiratory tract.

Pharmacokinetics

Bioavailability of oral budesonide in man is low (11-13%) due to an extensive first-pass metabolism in the liver.

The systemic availability of budesonide, with reference to the metered dose is 33%. In adults, the maximal plasma concentration after administration of 256 micrograms budesonide is 0.64 nM and is reached within 0.7 hours. The AUC after administration of 256 micrograms budesonide is 2.7 nmolh/L in adults.

Indication

Seasonal allergic & allergic/non-allergic perennial rhinitis. Treatment of nasal polyps. Preventive against nasal polyp after polypectomy.

Dosage and Administration

For nasal inhalation. Dosage should be individualized.

Rhinitis (Adults including elderly)

Recommended start dose	Once daily dosing	Twice daily dosing
256 micrograms per day	Two applications of 64 micrograms into each nostril each morning or If good effect is achieved, one application of 64 micrograms	One application of 64 micrograms into each nostril morning and evening

Nasal Polyps (Adults including elderly)

Recommended start dose	Once daily dosing	Twice daily dosing
256 micrograms per day	Not applicable	One application of 64 micrograms into each nostril morning and evening.

Treatment can be continued for up to 3 months.

Patients should be reminded of the importance of taking this medicine regularly. The dose should be titrated to the lowest dose at which effective control of symptoms is achieved.

Children: There are insufficient data to recommend the use of Budenase AQ 64 mcg Aqua in children. However, it is unlikely that the risk/benefit ratio in children is different from that in adults.

Contraindications

Budesonide aqueous nasal spray is contraindicated in patients with a hypersensitivity to any of its ingredients.

Warning & Precautions

Special care is demanded in treatment of patients transferred from oral steroids to Budesonide Aqueous nasal spray where disturbances of the hypothalamic-pituitary-adrenal (HPA) axis could be expected.

Special care is needed in patients with fungal and viral infections of the airways and in patients with lung tuberculosis.

The patient should be informed that the full effect of Budesonide Nasal Spray is not achieved until after a few days treatment. Treatment of seasonal rhinitis should, if possible, start before exposure to the allergens. Concomitant treatment may sometimes be necessary to counteract eye symptoms caused by the allergy. In continuous long-term treatment, the nasal mucosa should be inspected regularly e.g. every 6 months.

Systemic effects of nasal corticosteroids may occur, particularly at high doses prescribed for prolonged periods. Growth retardation has been reported in children receiving nasal corticosteroids at licensed doses.

It is recommended that the height of children receiving prolonged treatment with nasal corticosteroids is regularly monitored. If growth is slowed, therapy should be reviewed with the aim of reducing the dose of nasal corticosteroid, if possible, to the lowest dose at which effective control of symptoms is maintained. In addition, consideration should also be given to referring the patient to a paediatric specialist. Treatment with higher than recommended doses may result in clinically significant adrenal suppression. If there is evidence for higher than recommended doses being used, additional systemic corticosteroid

cover should be considered during periods of stress or elective surgery.

In vivo studies have shown that oral administration of itraconazole and ketoconazole (known inhibitors of CYP3A4 activity in the liver and in the intestinal mucosa, may cause an increase in the systemic exposure to budesonide. This is of limited clinical importance for short-term (1-2 weeks) treatment with itraconazole or ketoconazole, but should be taken into consideration during long-term treatment.

Special Precautions:

Fungal & viral nasal infections, chronic use(inspect nasal mucosa regularly). Pulmonary TB.

Interaction with other Medicaments

The metabolism of budesonide is primarily mediated by CYP3A4, a subfamily of cytochrome P450. Inhibitors of this enzyme, e.g. itraconazole and ketoconazole,

can therefore increase systemic exposure to budesonide. However, the use of itraconazole or ketoconazole concomitant with Budesonide Aqueous Nasal Spray for shorter periods is of limited importance.

Pregnancy and Lactation

Pregnancy

Administration during pregnancy should be avoided unless there are compelling reasons. Results from prospective epidemiological studies and from worldwide post marketing experience indicate no increased risk for overall congenital malformations from the use of intranasal budesonide during early pregnancy.

Lactation

Budesonide is excreted in breast milk. However, at therapeutic doses of Budesonide Nasal Spray no effects on the breast fed child are anticipated. Budesonide Nasal Spray can be used during breastfeeding.

Side effect

Adverse reactions, which have been associated with budesonide, are:

Immune system disorders	Uncommon	Immediate and delayed hypersensitivity reactions including urticaria, rash, dermatitis angioedema and pruritus
	Not Known	Anaphylactic reaction
Eye disorders	Rare	Raised intraocular pressure or glaucoma
Respiratory, thoracic and mediastinal disorders	Common	Haemorrhagic secretion and epistaxis Nasal Irritation (sneezing, stinging and dryness)
	Very rare	Nasal septum perforation Ulceration of mucus membrane

OVERDOSAGE

Acute overdose with Budesonide Nasal Spray should not present clinical problems. Inhalation of high doses of corticosteroids may lead to suppression of the hypothalamic-pituitary-adrenal (HPA) axis function

Storage

Store below 30°C. Protect from light.

Shelf life:

2 years

Presentation :

Carton containing unit pack of 120 metered dose.

Registration holder in Malaysia

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NASAL SPRAY

SAP Code: xxxxxxxx (Ver. 01)
Label size: 180 x 300 mm
Colours:
 CMYK

Reference : 21085393
2D code : xxxxxxxx
Country : Malaysia
Date: 04/01/2023

Path: OneDrive-Cipla Limeted\Ganesh\Malaysia\xxxxxxx
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